

Tiru ● REAL-TIME HEALTH FACILITY OPERATIONS INTELLIGENCE

TIRU

The operating system for what happens *inside* the hospital —
attendance, coverage, communication, and incidents, visible in real time
for the first time.

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AHIF 2026 FELLOW · ADDIS ABABA, ETHIOPIA

“Tiru” means good in Amharic – a byword for quality. It is our standard for the care, and for the operations, that every facility should deliver.

The unwatched engine room.

Private healthcare races to digitize the patient-facing experience — while the operational core stays invisible, manual, and unaccountable. These are not edge cases. They are daily events.

⊖ SPECIALIST LATENESS

The 8:00 AM specialist arrives at 10:00 AM — and nothing flags it.

A patient who was promised 8:00 **waits two hours**. The facility pays for hours not worked. No system raises an alert. Leadership only finds out when the patient complains — if they complain at all.

→ Cost: payroll loss + reputation + patient safety risk

⚠ PATIENT SAFETY

Midnight seizure. The emergency medication shelf is empty.

A patient arrives in status epilepticus. The night nurse reaches for the emergency anticonvulsant — **out of stock**. The stock-out was flagged two days earlier by a junior staff member who had no reliable channel to escalate it. The report stayed in someone's head.

→ Cost: patient harm, liability, institutional trust

📄 SERVICE FAILURE

Lab reagent outage. The patient already paid and waited.

A radiology system or lab reagent fails mid-morning. **No one notifies waiting patients** in time. They discover the outage at the window — after waiting, after paying. Frustration turns to a refund dispute and a damaged relationship the facility will spend months trying to repair.

→ Cost: refunds + churn + reputational erosion

● WHY THIS, WHY NOW

Built from inside.

Not a software team imagining hospital problems — a physician who lived them across five years of clinical practice, in the departments and at the desk where they actually happen.

- 01 **The conviction:** before a facility can serve the patient outside its walls, it must put its own house in order inside them. That is the sequence everyone is skipping.
- 02 **The defensibility:** these problems are invisible from outside the building — obvious only to someone who has stood at that desk.
- 03 **The advantage:** a physician-founder can walk into any private facility and be trusted as a peer — not a software vendor.

● TARGET CUSTOMERS

Built for the people who run the building.

Our primary buyer is the decision-maker who controls the budget and feels the pain most acutely — the owner, CEO, or General Manager of a private health facility.

PRIMARY · IMMEDIATE

Private Clinic & Hospital Owners

100+ mapped in Addis Ababa. Single-site to small chains. Monthly ETB subscription. Direct sales via Tiru Medical Directory pipeline.

SCALE · NETWORK DEALS

Multi-Site Healthcare Networks

Groups managing 3–20 facilities across one city or region. Network agreement covers all sites under one dashboard, at network pricing.

HARD CURRENCY · USD

NGOs & International Health Partners

Donor-funded programs needing verified attendance records for accountability reporting. USD-denominated MOU. Route to venture-scale revenue.

CORPORATE WELLNESS

Corporate & Institutional Clients

Companies running on-site medical teams, occupational health units, or shift-based clinical staff.



One platform. Every operational layer.

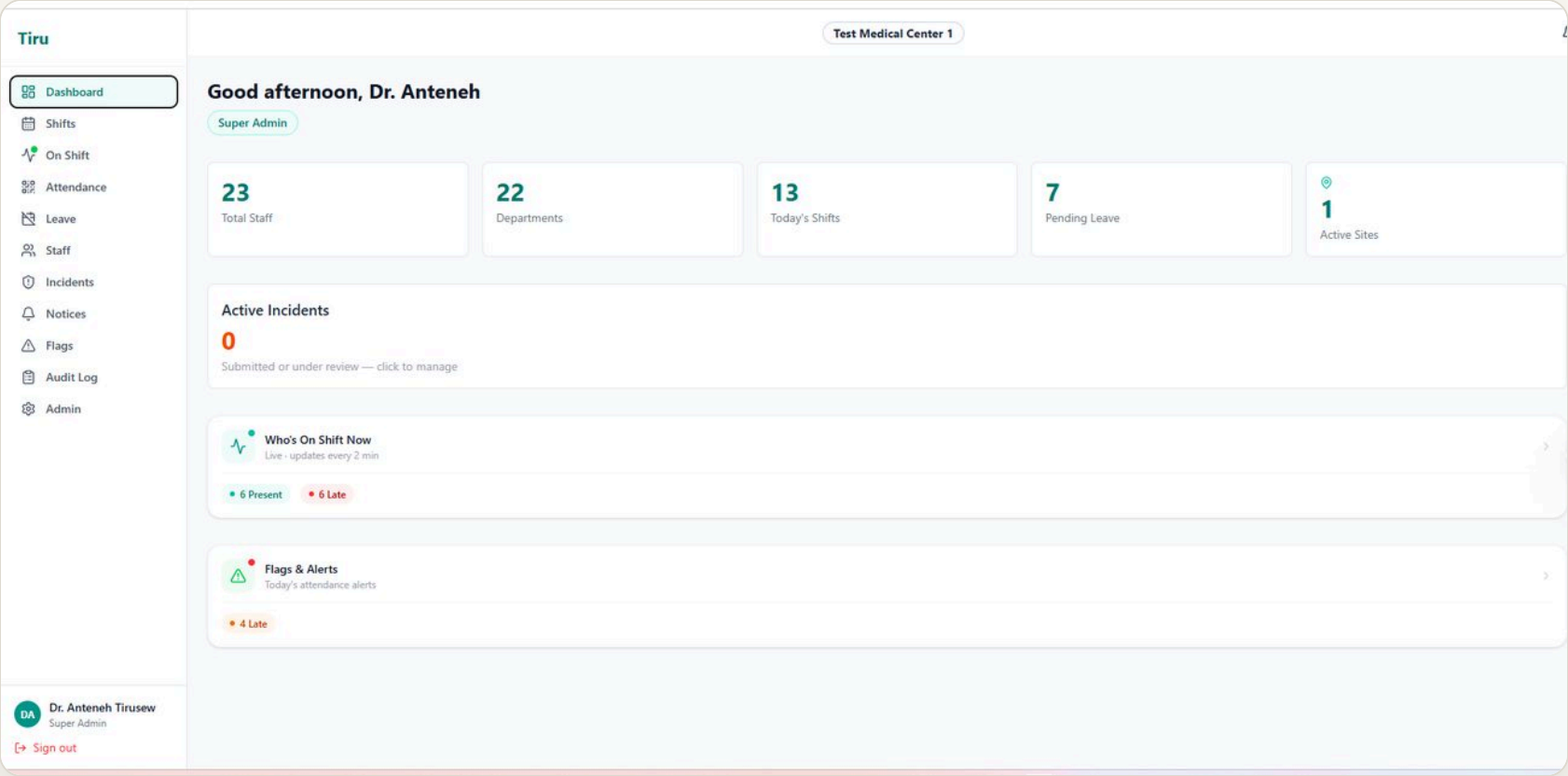
A single hybrid web-and-mobile platform built to a working MVP — functional end-to-end across every module.

- **Verified-presence attendance** — contactless QR + GPS geofence
- **Internal communication** — targeted & broadcast notices
- **Flags & alerts engine** — turns data into decisions
- **Audit log** — tamper-evident record of every action
- **Live shift & coverage view** — scheduled vs. present, real time
- **Incident reporting** — structured, anonymous-capable
- **Leave & approvals** — full digital lifecycle
- **Equity-first kiosk** — a full channel for every worker

THE PRODUCT — LIVE

The leadership picture, in real time.

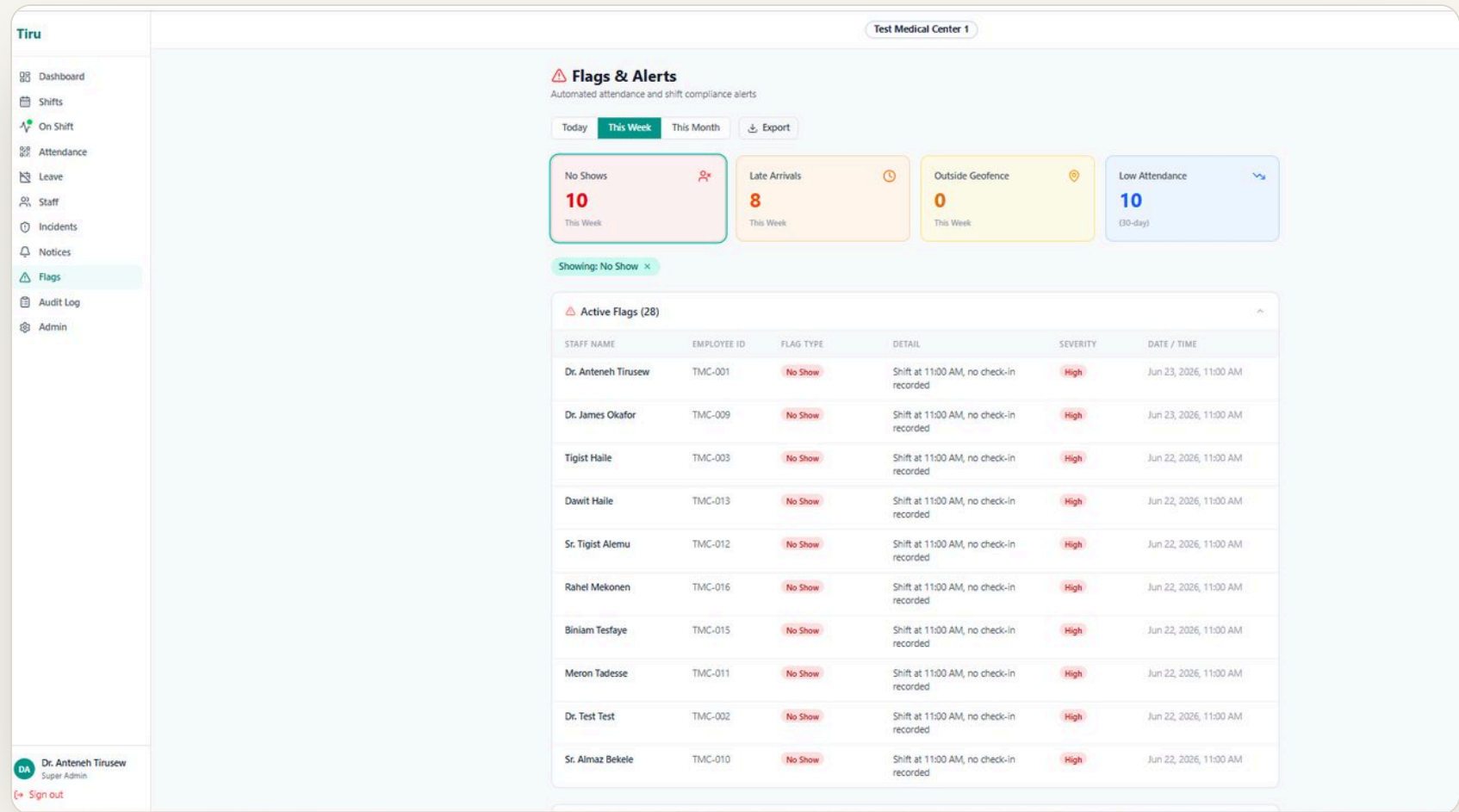
Total staff, departments, today’s shifts, pending leave, active sites, open incidents — one screen, updated live. The view no legacy attendance system can produce.



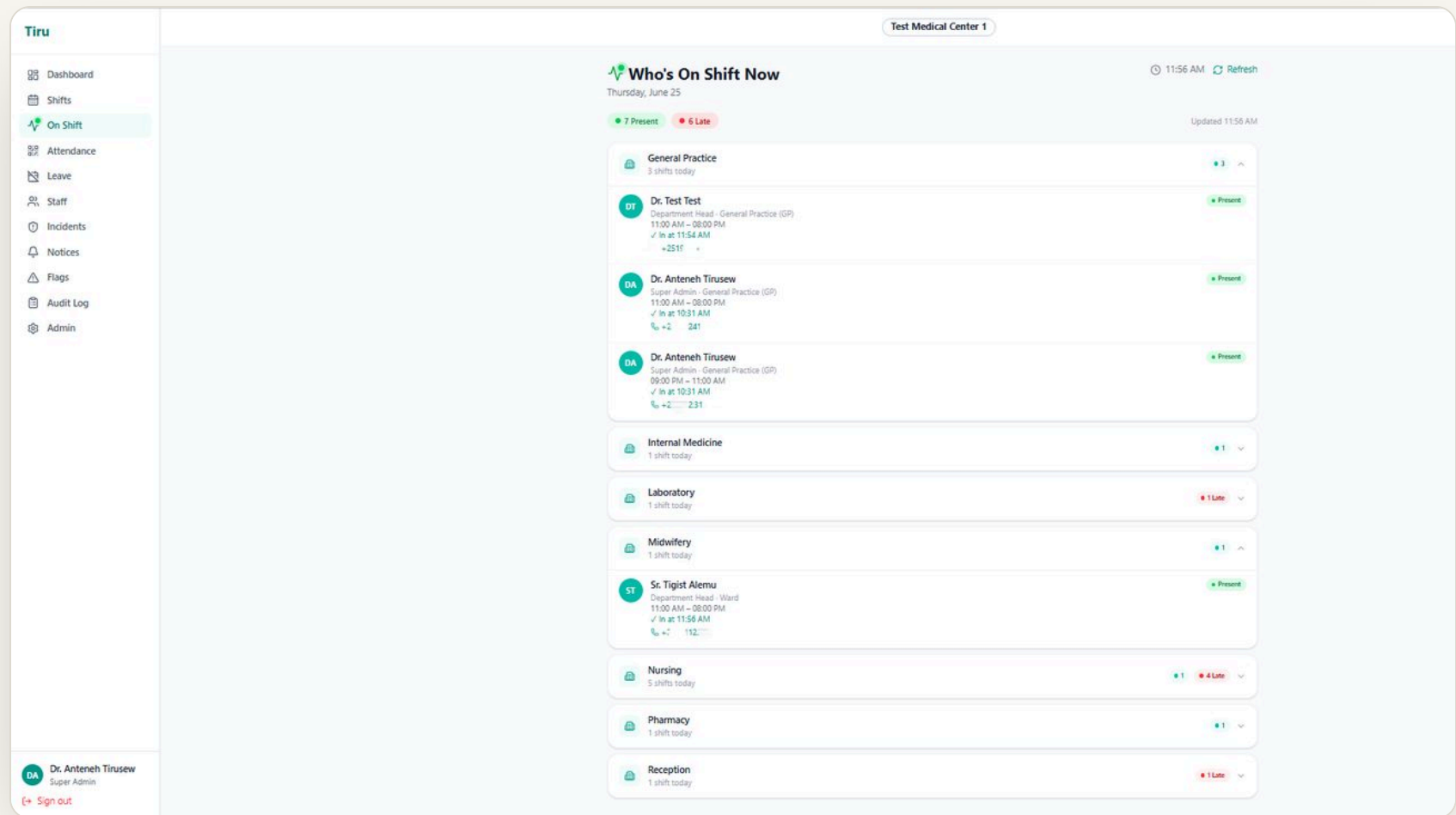
// LEADERSHIP DASHBOARD — LIVE OPERATIONAL STATE

Scheduled vs. present, at a glance.

Every department, cross-referenced against actual clock-ins. The flags engine then escalates no-shows, lateness, and outside-geofence attempts to the right person automatically.



// FLAGS & ALERTS — DATA INTO DECISIONS



// WHO'S ON SHIFT NOW — REAL-TIME COVERAGE

Presence, verified — not asserted.

Most systems ask “was a credential entered?” Tiru answers a far harder question: “is this enrolled person, on their own device, physically inside right now?”

LAYER 1 — LIVE TODAY

Rotating single-use QR + server-side clock + GPS geofence. Off-site clock-ins are refused outright.

LAYER 2 — IN DEVELOPMENT

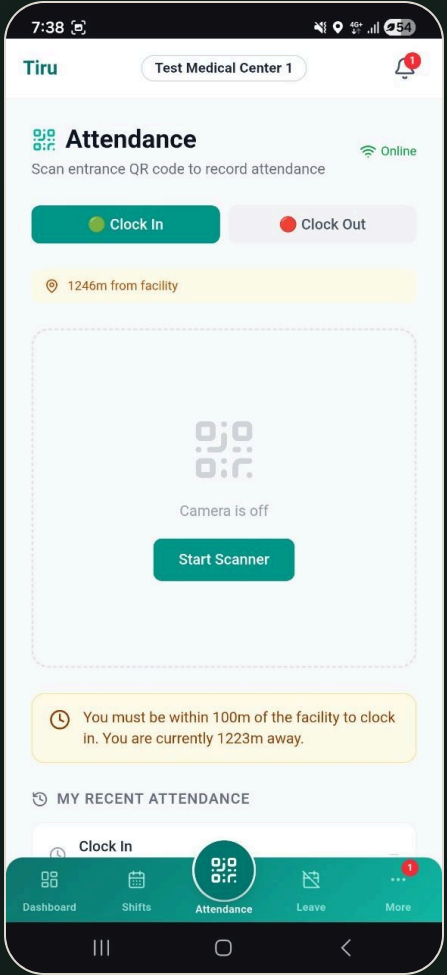
Device (hardware ID) + SIM (ICCID) binding. Single-device registry. Optional randomized 3×/week selfie match.

LAYER 3 — ON THE ROADMAP

AI face comparison + behavioral biometrics (PIN cadence) on the shared kiosk.

LAYER 4 — PHASE 3: COMPLIANCE ENGINE

Offline-first GPS pings every 10 minutes verify 8 cumulative hours of facility presence per shift — anchored to Ethiopia’s 48hr/week labour law mandate. Authorized lunch breaks excluded via a dedicated tap. A fingerprint tells you when someone arrived. Tiru tells you where they were all day.



// CLOCK-IN REFUSED — 1,246 m FROM FACILITY

We prove you were there — all day

Ethiopia’s 48hr/week mandate. Almost no facility can currently prove compliance. Tiru Phase 3 does.

HOW IT WORKS

BACKGROUND TRACKING

Background GPS ping every 10 minutes — offline-first, syncs on clock-out.

LUNCH BREAK

Lunch Break tap pauses monitoring for authorized breaks (90-min auto-expiry guard).

COMPLIANCE ENGINE

Server-side compliance engine: verified hours vs. 8hr mandate.

WHAT LEADERSHIP SEES

- 01 Per-person daily compliance card
- 02 Weekly 48hr mandate fulfilment report (exportable)
- 03 Real-time “Presence Deficit” alert before shift ends

A fingerprint tells you when someone arrived.
Tiru tells you where they were all day.

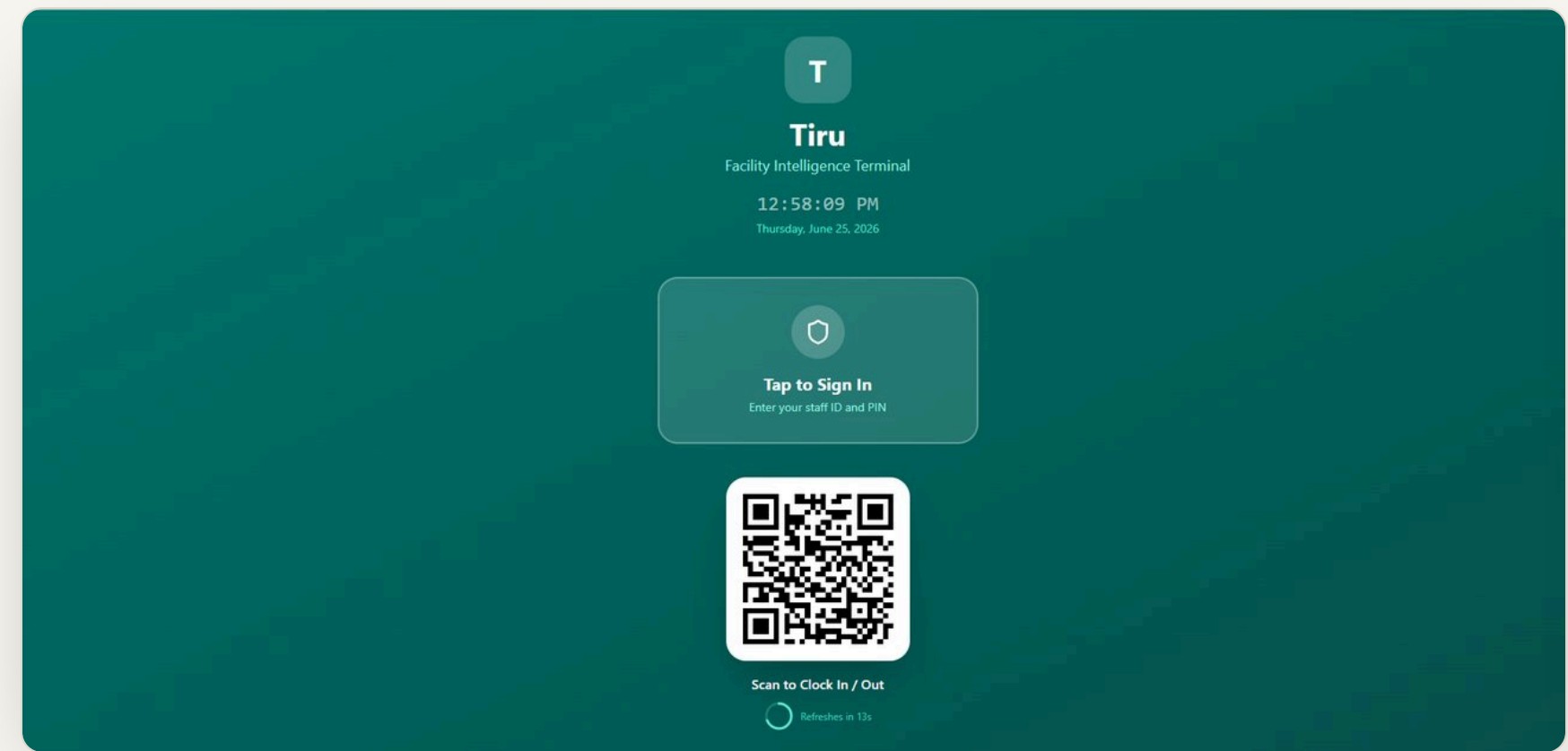
● WHY NOT FINGERPRINT?

Built for the clinic. Not retrofitted to it.

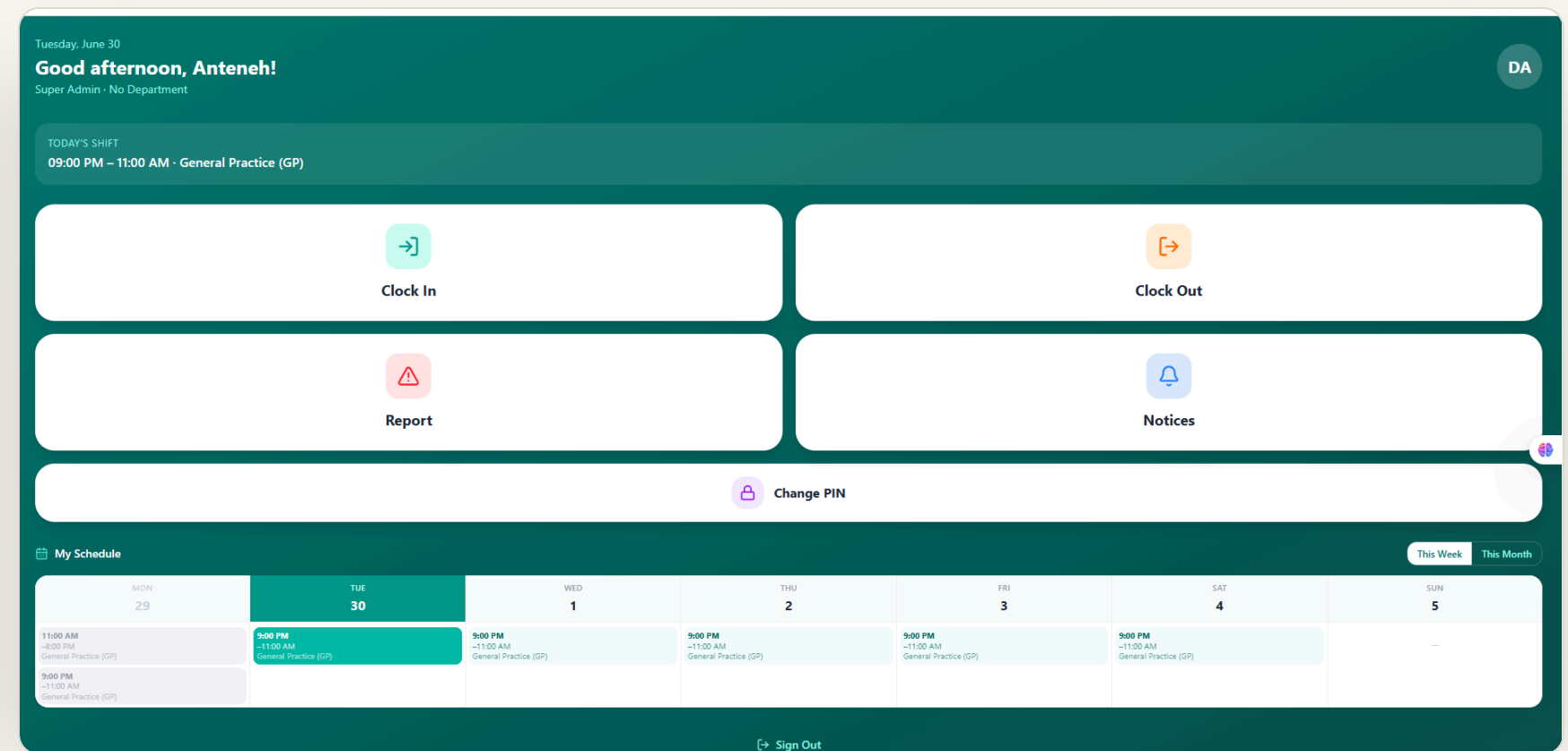
DIMENSION	● Fingerprint Biometric	● Tiru
REAL-TIME VISIBILITY	None Data sits on a local device or spreadsheet. Leadership sees yesterday.	Live CEO sees who is present, late, or missing right now — from anywhere.
HYGIENE / INFECTION RISK	High Shared-contact surface. MRSA, E. coli, Pseudomonas recovered from devices in peer-reviewed studies. Suspended during COVID-19.	Zero-contact QR scan — no shared surface, no cross-contamination risk in a clinical setting.
COST & HARDWARE PER SITE	High Device purchase, installation, and maintenance per site. Multiplies with every new location.	Near-zero A new site is a configuration record. No hardware, no installation, no per-site cost.
MULTI-SITE SCALABILITY	Poor Each site needs its own device, its own data silo. Networks get messier as they grow.	Native All sites on one dashboard. A network of 20 facilities costs the same to add as the second.
INTERPRETATION & ACTION	Manual Raw punch logs. Someone must export, reconcile, and interpret before a decision can be made.	Automated Flags engine converts data into alerts automatically — no-shows, lateness, geofence breaches escalated to the right person.
REMOTE MANAGEMENT	Impossible Physical access to the device required. No visibility outside the building.	Full Leadership monitors, audits, and communicates with any site from any device, anywhere.
STAFF EQUITY & ACCESS	Partial Requires physical presence at device. No mobile path. One point of failure per site.	Universal Smartphone app + equity-first kiosk (ID + PIN) — nobody excluded, including janitorial and security teams.
IDENTITY ASSURANCE	Biometric only Strong at a single point. No GPS, no device check, no replay protection.	Layered GPS geofence + rotating single-use QR + device binding + SIM binding + optional selfie match.
COMMUNICATION CHANNEL	None Attendance only. No notices, no incident reporting, no internal comms.	Built-in Direct CEO-to-all-staff push notifications — including janitorial and security — with no intermediary.
CONTINUOUS PRESENCE VERIFICATION	X Clock-in only Records arrival and departure. No verification during the shift. Staff can clock in and disappear undetected.	✓ Throughout the shift 10-minute GPS pings verify cumulative geofenced presence against the 8-hour labour law mandate. Lunch breaks excluded.

The kiosk is a full channel — not a fallback.

Staff without a smartphone clock in, receive notices, report incidents, and view the live schedule on a shared terminal. A CEO can reach the security or janitorial team directly — no intermediary, no message lost.



// FACILITY INTELLIGENCE TERMINAL — EMPLOYEE ID + PIN LOGIN — SELF-REFRESHING ENTRANCE QR — FULL NOTICE & INCIDENT CHANNEL



Staff dashboard — shift, attendance, and notices in one view.

● WHERE WE ARE

An MVP that already works.

Designed, built, and validated single-handedly by the founder — clinical insight plus hands-on engineering. Proof of conviction and capability.

MVP

built & deployed, functional end-to-end across every module

105+

private facilities mapped — [Tiru Medical Directory](#), a parallel sister project & our direct sales pipeline

2

co-founders — clinical insight (Dr. Anteneh Tirusew) + full-stack engineering (Yohana Mekuria)

AHIF

2026 Africa Healthcare Innovation Fellow — institutional validation

Per-staff SaaS. Zero per-site hardware.

Tiered subscription that scales with staff and facility count at near-zero marginal cost — the structural opposite of device-per-site biometrics.

DIRECT · ETB Single Facility One clinic or hospital, onboarded and live in days.	SCALE · ETB Healthcare Network A chain under one master agreement and unified dashboard.	HARD CURRENCY · USD NGO / International Donor-accountability MOUs — the route to venture-scale revenue.	LATER PHASE Government A dedicated public-sector framework.
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Beachhead: Addis Ababa private tier (100+ mapped) → East Africa → any shift-based workforce. Healthcare is the focused wedge.



What partnership unlocks.

The product is built. The next unlock is external validation — we are seeking either or both:

// Design partner facility

A private facility in Addis Ababa to run the first measured, baseline-to-impact deployment.

// Accelerator / investor

To fund external customer acquisition, a first USD NGO pilot, and the team to scale beyond the founder.

Early entry to a physician-founded, fellowship-validated venture solving a universal, unserved, and measurable problem — at maximum upside and minimum dilution.

● THE VISION

Every hospital in Africa will
run on real-time operations
data.

The only question is whose
platform.

Tiru begins where the problem is most acute and the proof is cleanest
— and expands into the full operating system for healthcare delivery.

Tiru.

